

osteosarcoma-mets-dll3-h7r2

Plain-language summary for patient and caregiver

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What this page is

You (or your clinician) asked Libby a narrow question. In metastatic osteosarcoma where there's a hint that DLL3 *and* PRAME might be present, what treatments would target either of those? You also said you'd consider high-risk options when the upside is meaningful, and the board read this report through that lens.

One thing matters before anything else. Both signals you gave us are at the RNA level. RNA tells us each gene is being read inside the cell. It doesn't tell us the actual protein is sitting where the new drugs need to find it. For DLL3 the drugs need DLL3 protein on the outside of the cell. For PRAME the drugs need PRAME protein inside the cell *and* a specific immune marker called HLA-A*02:01 on the patient (because PRAME-targeting drugs are designed only to work when that marker is present). So the recommendations come in two parts. First, a set of confirmation tests that everyone agreed should happen up front. Then, options that only open up depending on which test result comes back positive.

The page is also scoped tightly. It only covers drugs that target DLL3 or PRAME, because those were the question. Plenty of other treatments exist for relapsed osteosarcoma, and those are a conversation for your oncologist, not this report.

What we know about your cancer

You're in your late teens or twenties with metastatic (stage IV) osteosarcoma, a cancer that started in bone. The board assumed you'd had standard frontline chemotherapy (the MAP regimen: methotrexate, doxorubicin, cisplatin), since that's the default for this disease, but your inputs didn't say how you responded or whether you've had anything since. Your day-to-day function looks like ECOG 1, which roughly means you're up and around but tire more easily than you used to. The two features driving this report are the DLL3 and PRAME RNA signals, with the caveat from the section above.

What you told us matters most

You said you accept high-risk, high-reward options when the possible upside is meaningful. You didn't name any toxicities you wanted to veto outright, and you flagged a clear preference for clinical trials over off-trial experimental use. The board took those signals seriously. They're the reason first-in-disease trial options even made it onto the page.

The first step everyone agreed on

Before any treatment decision, get three tests done. They can run in parallel and the workup is the same one biopsy block plus one blood draw, so this is one trip, not three.

1. ****DLL3 protein test on the tumor (IHC stain using the SP347**
2. **PRAME protein test on the tumor (a different IHC stain).**
3. **HLA-A*02:01 typing on a blood sample.** Confirms whether your

In most cases your oncologist can order the IHC tests on stored tissue from a previous biopsy. If there isn't enough archived tissue, a fresh sample works too. None of these tests is toxic. Total turnaround is roughly one to three

weeks for the IHC work; the HLA typing is faster. The cost is trivial compared to a single treatment cycle.

All five reviewers put this in the rank-1 slot, and nobody dissented. The results decide which treatment options below are on the table at all. The two pathways are independent of each other, so you might have neither, one, or both.

If both DLL3 IHC *and* the PRAME / HLA pair come back negative, both pathways close and this page has no further treatment recommendations within scope. The conversation then shifts to standard relapsed-osteo options that your oncologist can take you through.

The options the board considered

There are four options, grouped by which test result has to be positive for them to be on the table. The DLL3 options come first because that was the original RNA signal in the case file; the PRAME options are equally serious if both PRAME tests are positive.

Option 1 — Tarlatamab via clinical trial NCT06788938 (DLL3 pathway)

Only available if the DLL3 IHC comes back positive. If the IHC is negative or below the trial's threshold, this option is off the table.

When the DLL3 test is positive, the board recommended tarlatamab as the lead option in the DLL3 pathway.

So what is it? Tarlatamab is a "bispecific T-cell engager." You get it as an infusion every two weeks. The drug works like a molecular clip. One end grips DLL3 on the cancer cell, the other end pulls in one of your own T cells to attack it. It's currently approved for small-cell lung cancer. NCT06788938 is a "basket" trial, meaning it accepts any DLL3-positive tumor (osteosarcoma included), provided the IHC confirms DLL3 is on the cell surface at high enough levels.

What might the upside look like? In small-cell lung cancer, where this drug is already approved, a recent randomized trial showed tarlatamab roughly cut the risk of death in half compared to chemotherapy. About 40 out of 100 patients had measurable tumor shrinkage. Whether any of that carries over to osteosarcoma is exactly the unanswered question this trial is designed to answer. I can't translate the lung-cancer numbers cleanly into an osteosarcoma expectation here, because no one has published that data yet and it would be misleading to pretend otherwise.

On the risk side, about half of patients get a reaction called "cytokine release syndrome." It's a flu-like immune flare; most cases are mild and roughly 1 in 100 are severe. About 1 in 10 patients have neurologic side effects. The first cycle requires inpatient monitoring, so you'd be in hospital while the step-up doses are given.

The board wasn't unanimous on this one. Four reviewers endorsed it in the positive-IHC scenario (the risk-taker, the advocate, the consensus reviewer, and the conservative). The conservative's earlier worry was specifically about treating an unconfirmed target; once IHC locks the target down, that worry goes away. The critic still dissented even with a positive IHC, on a single specific point: no osteosarcoma patient has been published as receiving tarlatamab yet. Whoever takes this trial is in the very first cohort. That's a real thing to weigh, not a

footnote.

Option 2 — SHR-4849 / IDE849 via clinical trial NCT07174583 (DLL3 pathway, considered with caveats)

Only available if the DLL3 IHC is positive AND the trial sponsor (IDEAYA) confirms osteosarcoma is on their basket eligibility list. That's two locks, not one.

Why include it at all? It's a different *kind* of DLL3 drug from tarlatamab. Tarlatamab uses your T cells; SHR-4849 is an "antibody-drug conjugate," which means the antibody finds the DLL3 target and delivers a chemotherapy payload directly to the cancer cell. If the tarlatamab path is foreclosed (for example, the cytokine reaction is too rough to continue, or the cancer adapts and stops showing DLL3), an ADC keeps the DLL3 strategy in play through a different mechanism.

Why "considered with caveats" rather than a flat recommendation? Two reasons. First, no clinical results have been published for SHR-4849 yet. The phase 1/2 trial is ongoing, so the efficacy and toxicity profile will get sharper over time, but right now we're working from mechanism rather than data. Second, the basket eligibility list for this trial doesn't yet say "osteosarcoma" out loud. Pursuing it means your oncologist (or you) reaching IDEAYA medical affairs directly to confirm fit before going further.

Two reviewers (the risk-taker and the advocate) endorsed it on the basket-trial principle and the second-mechanism hedge. The critic dissented on the lack of published clinical data. The conservative dissented on the eligibility uncertainty. Both dissents are real concerns to weigh.

Option 3 — IMA203 (PRAME-TCR-T) via clinical trial NCT03686124 (PRAME pathway)

Only available if both PRAME IHC AND HLA-A*02:01 typing come back positive. Both have to be positive together. PRAME without HLA, or HLA without PRAME, doesn't unlock this option.

When both PRAME tests are positive, the board ranked this as the lead option in the PRAME pathway.

What is it? IMA203 is a "TCR-T cell therapy." Your own T cells are collected, taken to a lab, given a custom receptor that recognizes the PRAME-derived peptide on HLA-A*02:01, *multiplied, and infused back into you.* *NCT03686124 (the Immatics ACTengine basket) accepts any PRAME+/HLA-A*02:01+ solid tumor, sarcoma cohorts included.*

What might the upside look like? The published cohort data so far covers 28 patients across various solid tumors, including some sarcomas. About 54 out of 100 had measurable tumor shrinkage. That's a strong signal in heavily pretreated cancers, with responses lasting at least nine months in the patients who responded. Whether osteosarcoma specifically responds the same way is something the trial is set up to answer.

On the risk side, this is one of the more demanding options on the page. Almost everyone gets cytokine release syndrome (most cases mild, but about 1 in 10 are severe). About 1 in 4 have neurologic side effects. Before the cells are infused, you get a short course of two chemotherapy drugs (Cy/Flu) to make room for the new T cells; that drops your blood counts for several weeks. One published patient died from a complication of that

pre-treatment chemotherapy. The manufacturing of your custom T cells takes four to six weeks, so this isn't a fast-start option.

The board split on this one. Three reviewers (the risk-taker, the advocate, and the consensus reviewer) endorsed it on the published efficacy and basket eligibility. The conservative dissented on the infrastructure and toxicity profile. The critic didn't object but didn't strongly endorse either.

Option 4 — IMC-P115C via clinical trial NCT07156136 (PRAME pathway, considered with caveats)

Only available if both PRAME IHC AND HLA-A*02:01 typing come back positive AND the trial sponsor (Immunocore) confirms osteosarcoma is on their basket eligibility list. Three locks.

What is it? Like IMA203, IMC-P115C targets the PRAME peptide on HLA-A*02:01, but it does so with a smaller, off-the-shelf molecule called an "ImmTAC" rather than your own engineered T cells. You get it as a weekly infusion. The same broader drug class includes tebentafusp, which is an approved drug for a rare eye cancer, so the mechanism class itself is validated. IMC-P115C specifically is in phase 1 testing right now, so we don't yet have published efficacy numbers for it.

Why "considered with caveats" rather than a flat recommendation? Same shape as Option 2. No published clinical data for this specific drug yet, and the basket's osteosarcoma eligibility hasn't been spelled out, so pursuing it means contacting Immunocore medical affairs first.

Two reviewers (the risk-taker and the advocate) endorsed it on the mechanism-class hedge (in case the TCR-T path in Option 3 isn't reachable). The critic dissented on the lack of published clinical data. The conservative dissented on the eligibility uncertainty.

What the board could not figure out

A few things the inputs didn't tell us, which would sharpen the plan:

- Were DLL3 and PRAME measured at the protein level (IHC) or only at
- What treatment have you actually had? The recommendations assume
- What's your day-to-day function (performance status)? Most trials
- Where do you live, and how reachable are academic cancer centers?

Questions to ask your oncologist

1. Can we order all three tests at once: DLL3 IHC (SP347), PRAME IHC,
2. Once the test results are in, can we walk through what each
3. If DLL3 is positive, what's the realistic path to NCT06788938 from
4. If PRAME and HLA are both positive, what's the realistic path to
5. If both pathways are foreclosed (negative on all three), what

6. What's your hospital's protocol for cytokine release syndrome and
7. For the two trials that say "considered with caveats" (Options 2
8. Have I already had any of the standard 2L+ osteosarcoma options?
9. Is there anything in my labs or imaging that would change which

Where to read more

- [Clinician summary](#). The technical version of this page.
- [Trial table](#). The trials Libby reviewed.
- [Evidence list](#). The clinical and lab studies cited.
- [Tumor-board transcript](#). What each reviewer actually said.
- [Recommendations detail](#). Full structured tables.

Sources

The recommendations cited the following published reports and clinical-trial records:

- Ahn et al. *N Engl J Med* 2023. Tarlatamab DeLLphi-301 in SCLC
- Mountzios et al. *N Engl J Med* 2025. Tarlatamab DeLLphi-304 in SCLC
- Wermke et al. *Lancet Oncol* 2024. IMA203 PRAME-TCR-T cross-tumor
- Hamid et al. *Cancer Discov* 2024. Brenetafusp PRAME ImmTAC class
- Iura et al. 2017. PRAME expression frequency in osteosarcoma
- Yao et al. *Oncologist* 2022. DLL3 as a therapeutic target review
- Wang et al. *J Exp Clin Cancer Res* 2018. Notch3/DLL3 axis in
- Trial registry NCT06788938 (UCCC-01 / UCLA L-10 tarlatamab basket).
- Trial registry NCT07174583 (IDEAYA SHR-4849 / IDE849 pan-tumor DLL3
- Trial registry NCT03686124 (Immatics ACTengine pan-solid PRAME
- Trial registry NCT07156136 (Immunocore IMC-P115C pan-tumor PRAME